Cross-District Care in Telangana

A descriptive analysis of 2015 health-claim records

RESEARCH QUESTION

How often did the hospital district differ from the patient's district in 2015, and how did those records vary by treatment category, hospital type, and recorded claim amount?

AT A GLANCE

DIFFERENT DISTRICT

55.28%

100,619 records

SAME DISTRICT

44.72%

81,399 records

Share of all 2015 records

WHAT THE RECORDS SHOW

Treatment volume: Medical Oncology is the largest cross-district category, with 22,769 records (22.6% of cross-district records).

Hospital type: Corporate facilities account for 69.5% of cross-district records. Their average recorded claim amount is 27,310, compared with 21,200 for Government facilities.

Reading the pattern: These are descriptive differences in the recorded data. The fields do not state why a patient received care outside their district.

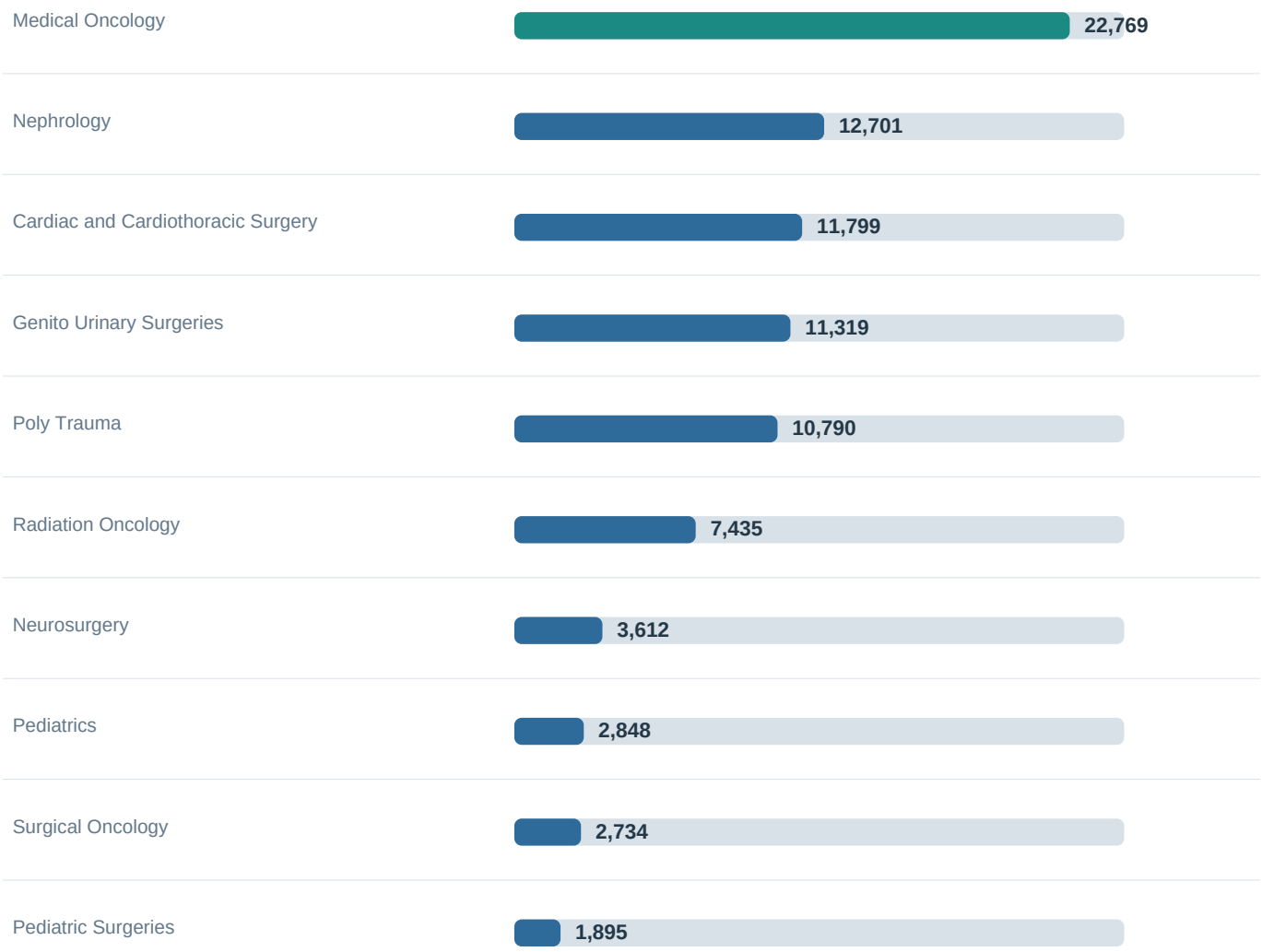
SCOPE

182,018 spreadsheet rows; every Claim Date is in 2015. Each row is counted as a record, not as a distinct patient. Claim Amount units/currency are not specified in the source.

Which treatments appear most often?

Cross-district records only. Bars show record counts by treatment category; the full Tableau view includes all 27 categories.

TOP 10 CATEGORIES BY RECORD COUNT



CONCENTRATION

The top three categories total 47,269 records, or 47.0% of all cross-district records. Medical Oncology alone contributes 22,769 records (22.6%). This shows where the record volume is concentrated; it does not establish a reason for patients' district-crossing care.

HOW TO READ THIS CHART

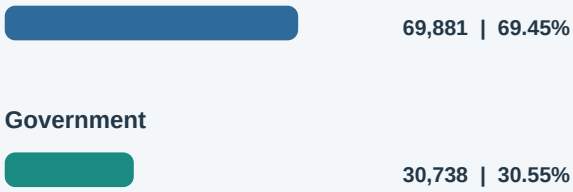
The comparison is among categories within the 100,619 records classified as Different district. Counts are not adjusted for the total number of claims in each category across all district statuses.

Volume and average amount by hospital type

Both comparisons use the 100,619 cross-district records. Count share and average claim amount answer different questions.

RECORD MIX

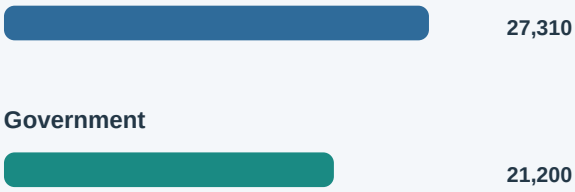
Share of cross-district records



Corporate accounts for the larger share of cross-district rows in this dataset. This is a share of cross-district records, not the cross-district rate within each hospital type.

AVERAGE CLAIM AMOUNT

Source units; currency not specified



Average recorded Claim Amount per row, not total amount paid. The Corporate average is about 6,110 higher in source units (about 28.8% above Government).

INTERPRETATION

The first chart describes the composition of cross-district records by facility type. The second compares average recorded amounts for those records. Neither chart shows that facility type caused patients to cross district boundaries, and the count split alone should not be read as a utilization rate.

KEY COMPARISON

69,881
Corporate cross-district records

30,738
Government cross-district records

27,310 vs 21,200
Average amount by type

The amount comparison uses the same record population as the count comparison. Values are rounded to the nearest source unit.

Average amounts vary by treatment

The highest averages are useful flags for review, but the number of records behind each average matters.

HIGHEST AVERAGE CLAIM AMOUNTS AMONG CROSS-DISTRICT RECORDS

Treatment category	Average amount*	Records
Cochlear Implant Surgery	**520,000**	**29**
Cardiac and Cardiothoracic Surgery	66,164	11,799
Surgical Gastro Enterology	53,426	298
Neurosurgery	49,613	3,612
Pediatric Surgeries	42,032	1,895
Critical Care	37,998	180
Surgical Oncology	34,248	2,734

SMALL-SAMPLE CAUTION

Cochlear Implant Surgery has the highest average recorded amount (520,000), but it is based on only 29 records. Its average should not be treated as typical for all cross-district care or compared without considering the much larger record counts in other categories.

* Claim Amount units and currency are not specified in the cleaned source file.

METHOD AND LIMITS

Scope: 182,018 rows; every Claim Date falls in 2015. No patient-district, hospital-district, or Claim Amount values were missing in the source rows checked.

District status: Patient and hospital district names were compared after trimming spaces and ignoring capitalization. Equal names were labeled Same district; unequal names Different district.

Amount summaries: Averages are calculated across records within each category or hospital type. They are not sums or amounts per unique patient. No currency label is provided in the data.

What cannot be concluded: The dataset does not record referral reasons, travel distance, local service availability, or patient preference. This analysis can identify patterns and possible topics for follow-up, but it cannot prove why care occurred outside a patient's district.

CONCLUSION

In 2015, 55.28% of recorded claims were associated with a hospital in a different district. Cross-district records were concentrated in a small set of treatment categories and more often recorded at Corporate facilities. These are descriptive patterns, not evidence of the underlying cause.